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Northgate Valley Clinic

Discharge Summary

Patient: Teodor Blascovich | **DOB:** 1971-11-02 | **MRN:** NVC-3310884 | **Admitted:** 2026-08-24 | **Discharged:** 2026-08-30 | **Ward:** Medical B | **Consultant:** Dr. Priyanka Ollerenshaw

Reason for admission

Six-day history of productive cough, fever to 38.9 C at home and progressive breathlessness on minimal exertion. The patient attended the emergency department after an episode of pre-syncope while climbing stairs. No chest pain, no haemoptysis, no calf swelling.

Background

Type 2 diabetes mellitus diagnosed 2014, managed with metformin. Hypertension. Ex-smoker, approximately 20 pack-years, stopped 2019. No known drug allergies. Lives with a partner in a first-floor flat, independent in all activities of daily living before this admission.

Findings on admission

Temperature 38.4 C, heart rate 104 and regular, blood pressure 128/74, respiratory rate 24, oxygen saturation 91 percent on room air. Coarse crackles and reduced air entry at the right base. Chest radiograph showed right lower lobe consolidation without effusion. White cell count 14.6 x10^9/L, C-reactive protein 188 mg/L, creatinine 96 umol/L. CURB-65 score 1.

Progress in hospital

Treated as community-acquired pneumonia with intravenous co-amoxiclav and oral clarithromycin, supplemental oxygen via nasal cannulae at 2 L/min and intravenous fluids for the first 36 hours. Blood cultures were negative at 48 hours; pneumococcal urinary antigen was positive. Oxygen was weaned off on day three and antibiotics were switched to the oral route on day four once the patient had been afebrile for 24 hours. Capillary glucose rose to 16.8 mmol/L during the acute phase and settled on the usual metformin dose without needing insulin. Physiotherapy reviewed the patient twice and confirmed independent mobility on the flat and up a flight of stairs before discharge. Repeat C-reactive protein on day five was 46 mg/L.

Condition at discharge

Afebrile for 72 hours, saturations 96 percent on room air at rest and 94 percent after climbing one flight, mild residual exertional breathlessness and an occasional dry cough.

Discharge medication

Drug	Dose	Frequency	Duration
Amoxicillin	500 mg	three times daily	5 days to complete
Clarithromycin	500 mg	twice daily	3 days to complete
Metformin	1 g	twice daily	continue long term
Ramipril	5 mg	once daily	continue long term
Atorvastatin	20 mg	once daily at night	continue long term
Paracetamol	1 g	up to four times daily	as required, 7 days

Follow-up and advice

Review in the general practice surgery within one week for a clinical check and repeat blood pressure. Repeat chest radiograph at six weeks to confirm resolution of the consolidation, booked for 2026-10-11. Pneumococcal and seasonal influenza vaccination to be offered in primary care once fully recovered. The patient has been advised to return to the emergency department if fever recurs, breathlessness worsens at rest, or new chest pain develops. A copy of this summary has been sent to the registered general practitioner.